

between healthcare and tourism ministries. Countries like Turkey and Thailand have gained significantly on the back of better perception,” he says.

Following the Covid-19 pandemic, the Turkish government funded marketing campaigns of companies in the country. Soon, it became a popular destination for cosmetic treatment and hair transplants. Thailand, meanwhile, is about 1.5-2x more expensive than India but remains a big market, since the process there is more organised. “India has a real opportunity to become the complex care capital of the world,” says Jain.

The organised market for medical concierge platforms has 5-10 players, and between themselves, they account for 800-1,000 patients every month. Of this, Jain’s company accounts for 70-80 patients. According to him, the unorganised market deals with one million patients annually or around 100x more than the organised segment. The volume is driven by an estimated 2,00,000 agents, with half coming from the Delhi-NCR region due to a larger corporate hospital network and easier connectivity from international destinations. By contrast, Mumbai has fewer hospitals (most of them are standalone), limiting options.

“The next priority should be co-ordinated execution. Medical value travel involves not only healthcare but external affairs, tourism, civil aviation, home affairs and AYUSH. A national medical value travel mission could align these stakeholders around medical diplomacy, visas, air connectivity, quality standards and international promotion,” says the Apollo Hospitals’ spokesperson.

Though Bangladesh sends the largest number of patients, Jain took the tougher route by targeting the UK market, where waiting times are longer and limited expansion of



“If we want to sustain double-digit growth over the next decade, we need an ecosystem that makes India the most trusted place in the world to seek medical care”

KARTHIK RAJAGOPAL
GROUP CHIEF OPERATING
OFFICER, MANIPAL HEALTH
ENTERPRISE



“Medical tourism falls between healthcare and tourism ministries. Countries like Turkey and Thailand gained on the back of better perception”

SAHIL JAIN
CEO & CO-FOUNDER, THE
MEDICAL TRAVEL COMPANY

medical infrastructure makes access challenging. “Last year, only 2% of the patients came from developed markets. Given healthcare inflation in the UK and the US, that should be a lot higher,” says Jain. The normal fee for anyone bringing in a patient is 25-30% of the total bill. An organised market gives hospitals access to patients from more countries and a more meaningful way to spend on marketing.

Like any other business, there are periods of uncertainty and Crisil Ratings’ Sethi thinks the West Asia crisis is a genuine challenge. “In the initial phase, there was a decline in the number of patients from Oman, Saudi Arabia, Iran, Iraq and Yemen due to frequent airport shutdowns and high airfares,” he says.

The inflow from the region is generally concentrated in the highest-realisation lines—oncology, transplants and complex cardiac—

and is heavily sovereign-sponsored. Fortis Healthcare’s Garg is clear that the future of India’s medical value travel industry will be defined less by cost arbitrage and more by clinical excellence, innovation, patient outcomes, trust and global partnerships. “As healthcare systems across the world grapple with rising costs, workforce shortages and growing demand, India has a unique opportunity to emerge as a preferred destination for advanced, specialised and accessible healthcare. The next decade could well be India’s most significant growth phase in international healthcare,” she says.

Rising healthcare costs globally present a significant opportunity for India. If executed well, the country can strengthen its position in medical tourism and build a durable competitive advantage. **BT**

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